3.9 Pneumothorax and Chest Drain*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I felt sudden chest pain and could not breathe." **Objective** • No breath sounds on one side • Chest moves unevenly • SpO_2 88%, RR 30/min	**ACTUAL** Ineffective breathing pattern related to lung collapse as evidenced by dyspnea, absent breath sounds and hypoxemia.	• Lung re-expanded and SpO_2 94% or more within 24 hours.	• Give oxygen as prescribed. • Assist with insertion of the chest drain. • Keep the drainage system below chest level and the tubing free of kinks. • Watch the water seal for swinging (tidaling) and bubbling. • Report tracheal deviation, hypotension or worsening dyspnea at once, as these suggest tension pneumothorax.	• Goal reached when the X-ray shows re-expansion and SpO_2 is on target.
Subjective • "The tube hurts when I move." **Objective** • Guards the drain site • Reluctant to move	**ACTUAL** Acute pain related to the chest drain and pleural irritation as evidenced by guarding and reported pain.	• Pain at or below 3/10 and joining in breathing exercises.	• Give analgesia regularly as prescribed. • Support the tubing so it does not pull. • Encourage deep breaths and shoulder exercises on the affected side.	• Met when exercises are done with bearable pain.
Subjective *No specific complaint* **Objective** • Chest drain in place	**RISK** Risk for physical injury as evidenced by risk factors: accidental disconnection or dislodgement of the chest drain.	• Drain stays secure and working.	• Check that every connection is tight. • Do not clamp the drain unless ordered. • Keep sterile gauze and clamps at the bedside. • If it falls out, cover the site with an occlusive dressing and call the physician immediately.	• Achieved if the drain remains in place.

*References for this care plan: [2], [17], [35], [37], [128]. Full entries are listed in the References section of the CPMN book.